National Cancer Control Programme (NCCP) 00720 · clinician draft

Neratinib · extended adjuvant treatment

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Weeks 1–2 · early diarrhoea prevention

Active medicines: Neratinib

Emetogenic risk: Oral minimal-to-low

Medicine	Dose / route	When
Loperamide	4 mg orally three times daily	Weeks 1–2 from the first neratinib dose National Cancer Control Programme (NCCP)-specific oncology prevention regimen; individual review is required.

Additional phase-specific support

- Start with the first neratinib dose. Titrate under the prescribed plan toward 1–2 bowel movements/day.
- Review constipation, hydration, electrolyte disturbance and diarrhoea severity promptly.
- No generic PPI order: review acid-suppressing medicines and interactions.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____

Prescriber / date: _____ Pharmacy review: _____

NCCP 00720 v2 · p5 · NCCP oral antiemetic guidance v6, Tables 5 and 8

Sources checked 2026-10-25 · SACTCheck v0.84.19 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Weeks 3–8 · continuing diarrhoea prevention

Active medicines: Neratinib

Emetogenic risk: Oral minimal-to-low

Medicine	Dose / route	When
Loperamide	4 mg orally twice daily	Weeks 3–8 Review bowel pattern, hydration and constipation.

Additional phase-specific support

- Titrate under the prescribed plan toward 1–2 bowel movements/day.
- New or worsening diarrhoea may require investigation, fluids and a neratinib hold; do not simply escalate loperamide indefinitely.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____

Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00720 v2 · p5 · NCCP oral antiemetic guidance v6, Tables 5 and 8

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Weeks 9–52 · symptom-directed bowel support

Active medicines: Neratinib

Emetogenic risk: Oral minimal-to-low

Medicine	Dose / route	When
Loperamide	4 mg orally as needed	Weeks 9–52 Prescriber must specify daily maximum and escalation instructions; source gives PRN dosing without a separate maximum here.

Additional phase-specific support

- Review severity, hydration and need to hold neratinib if diarrhoea occurs.
- Rescue antiemetics are PRN and selected after interaction/QT review.
- Confirm acid-suppression timing and the ongoing liver-test schedule.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____

Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00720 v2 · p5 · NCCP oral antiemetic guidance v6, Tables 5 and 8

Sources checked 2026-10-25 · SACTCheck v0.84.19 · Regimen-level drafts; prescriber and oncology-pharmacy verification required